MINDSET HEALTHCARE

Go-to-Market & Operations Strategy

Updated to incorporate the validated consultation, payment, and platform infrastructure build

This document supplements the business plan and pitch deck. Figures marked as illustrative are placeholders pending validation against real pilot data, and should be reviewed alongside the financial plan before distribution to investors.

1. Organizational Roles & Management Structure

Mindset Healthcare's technology and operations function is organized around two core mandates: designing and building the technology platform, and running day-to-day service delivery through Marketing and Referral roles.

1.1 Core Functional Roles

- **Marketing** — brand building, campaign execution, and channel partnerships.
- **Referral** — managing the pipeline of patient referrals into partner hospitals and providers.

1.2 Management Functions

- **Planning** — roadmap and resourcing across the platform and provider network.
- **Budget** — cost control and spend allocation across functions.
- **Forecasting** — demand and revenue projections used to guide hiring and marketing spend.
- **Referrers** — relationship management with the clinicians and partners who refer patients.
- **Clinician** — clinical oversight and quality assurance across the provider network.

1.3 Departmental Structure

Beneath management sit two departments — Finance and Marketing — each with its own reporting lines:

- **Finance** — Accounts and POS (point-of-sale) machines, covering payment processing and reconciliation.
- **Marketing** — Branches, split into internal and external positioning models, covering how the brand is presented to internal stakeholders (providers, staff) versus the external market (patients, diaspora, partners).

The founder/team org chart in the source notes references named individuals and an acceptance date (18th) that should be confirmed and inserted here before this section is finalized.

2. Client Acquisition Build-Out

The client-facing (patient-side) go-to-market motion centers on a lightweight digital funnel supported by a time-boxed paid campaign, channel-specific social media strategies, and organic outreach to clinicians.

2.1 Digital Funnel

- Website with a clear value proposition and calls to action.

- Landing page with an integrated lead capture form.
- Social media presence to drive awareness and traffic into the funnel.

2.2 Launch Campaign

An initial paid social media marketing campaign is planned to run over a 4-week period.

- **Duration:** 4 weeks.
- **Budget:** ₦5,000 weekly for the 4-week period (illustrative — to be validated against actual channel costs and CAC targets).

The budget figure above is a placeholder carried over from planning notes and should be reconciled with the bottom-up financial assumptions in the business plan.

2.3 Patient Application Funnel — Detailed Structure

Every channel feeds into the same underlying funnel, taking a prospective patient from first exposure through to a booked consultation and long-term retention. With the tiered consultation model now live in the backend, the Application and Booking stages carry a concrete, testable promise: a patient can name their urgency (Normal, Priority, Super Priority) and see it acted on end-to-end.

- **Awareness —** prospect sees a paid ad, organic post, or referral mention.
- **Landing Page —** clicks through to a page stating the value proposition (dual physical + mental health access) with a single clear call to action.
- **Application —** completes a short lead form: name, contact, location, stated need (physical, mental health, or both), and — now that tiering is supported — preferred urgency level.
- **Screening —** care coordinator reviews the submission for urgency and eligibility, and reaches out within a set response window.
- **Provider Match —** patient is matched to the right provider in the network, physical or mental health, based on need and location.
- **Booking —** appointment is scheduled directly with the matched provider or hospital, or — for consultations placed through the platform — a request is created and broadcast for a doctor to claim (see Section 3.3).
- **Consultation —** care is delivered, either in person or via the platform's referral pathway into a partner hospital, or as a live voice/video consultation through the platform itself.
- **Retention —** post-visit follow-up, satisfaction check, and prompts for repeat use or referral to family/friends.

Each stage should carry a target conversion rate once pilot data is available, so drop-off can be diagnosed stage-by-stage rather than only at the top and bottom of the funnel.

2.4 Channel Campaign Portfolios — Dual-Role Strategy

Every channel in the plan is asked to do two jobs at once: acquire patients directly, and recruit the clinicians and hospitals who refer patients in. Facebook, WhatsApp, Telegram, and LinkedIn each carry two campaign portfolios — Portfolio A for patient acquisition, Portfolio B for clinician/referrer outreach — with its own audience, content plan, funnel, and KPIs. Role-B portfolios share a red funnel treatment throughout this section so the referrer-recruitment thread is easy to trace across every channel. Section 6 below details how the newly built consultation, claim, and payment infrastructure sharpens the messaging in both portfolios.

3. Product & Technical Capability Layer

This section documents what has been built and validated in the platform backend since the last version of this strategy. It exists to keep go-to-market planning honest: every claim made to patients, doctors, hospitals, or investors in Sections 2, 4, 5, and 6 should trace back to a capability described here.

3.1 System Architecture Overview

The platform is organized into five layers, from the interfaces patients and doctors touch directly down to the operational tooling that keeps the system trustworthy.

- **Patient/Doctor App Interaction Layer —** registration, consultation requests, the doctor claim interface, and receipt of real-time/push updates.
- **Core Medical Workflow Backend —** the consultation state machine, tier logic, atomic doctor-claim locking, and mid-consultation escalation.
- **Financial Transaction Orchestration —** payment hold, capture, and release across the consultation lifecycle.
- **Compliance & Retention Operations —** biometric verification hooks, audit trails, and data retention rules aligned to NDPR expectations.
- **Monitoring & Operational Visibility —** health checks, timer jobs (including the 2-minute claim window), and alerting for operational SLAs.

3.2 Three-Tier Consultation Model

Patients now choose a consultation tier at the point of request, each mapped to a distinct communication mode and price point:

Normal (Voice-Text)	₦1,000 — asynchronous or text-based consultation for lower-urgency needs.
Priority (Voice Call)	₦5,000 — live voice consultation for moderate-urgency needs.
Super Priority (Video Call)	₦10,000 — live video consultation for high-urgency or complex needs.

Tier pricing is a product decision already reflected in the backend, not a marketing placeholder — but it should still be validated against Nigerian willingness-to-pay data and reconciled with the CAC and revenue assumptions in the financial plan before it is presented to investors as fixed pricing.

3.3 Doctor Claim Workflow

Consultation requests are broadcast to eligible doctors and claimed on a first-come basis, in an Uber-style model: whichever doctor accepts first is atomically assigned the case, and any later claim on the same request returns a conflict rather than a double booking.

- Doctors have a 2-minute window to accept a broadcast request before it is released back to the pool.
- Claims are atomic at the database level — two doctors cannot both claim the same request, and the second attempt is rejected with a conflict response rather than silently overwritten.
- A successful claim moves the consultation from **REQUESTED** to **DOCTOR_ACCEPTED** and triggers real-time and push notifications to the patient (see Section 3.5).

3.4 Consultation Lifecycle & State Machine

Every consultation moves through a fixed sequence of states, giving both the product and the operations team an unambiguous source of truth for where a case stands:

- **REQUESTED —** patient submits a tiered request; a payment hold is created against the tier price.
- **DOCTOR_ACCEPTED —** a doctor claims the request within the acceptance window.
- **IN_PROGRESS —** the consultation session is live; the tiered flow supports mid-consultation escalation if a patient's need turns out to require a higher tier (e.g. a Normal text chat that needs to become a Priority call).
- **COMPLETED —** the session ends and the payment hold is captured.

3.5 Payment Hold, Capture & Release Lifecycle

Payment is orchestrated around the consultation lifecycle rather than charged upfront outright, which protects both patients (no charge without care delivered) and doctors (funds are already secured before they accept a case).

- A payment hold is placed against the patient's chosen tier at the moment a request is created (REQUESTED).
- The hold is captured when the consultation reaches COMPLETED.
- If a request goes unclaimed within the acceptance window, or is cancelled before completion, the hold is released rather than captured.

3.6 Real-Time Communication & Push Notification Infrastructure

Real-time and push pathways are wired into the two events that matter most to a patient waiting on care:

- Request created — the patient receives confirmation their request is live and broadcasting to doctors.
- Doctor accepted — the patient is notified in real time the moment a doctor claims their case, closing the anxious gap between requesting help and knowing someone is coming.

This infrastructure is the backend foundation for any GTM claim about speed-to-care (see Section 6.1) and should be the basis for the one SLA number patients are told to expect, once pilot data confirms a realistic acceptance-window fill rate.

3.7 Biometric Verification Hooks

Face and speaker verification hooks are integrated at the compliance layer, intended to confirm doctor identity at claim time and/or patient identity at intake, supporting trust and fraud-prevention claims made to hospitals, regulators, and investors. The current scope is integration hooks rather than a fully enforced verification gate — the rollout plan (mandatory vs. optional, which flows require it) is an open item (Section 7).

3.8 Operations Baseline

The monitoring and operational layer keeps the tiered, real-time system observable and recoverable:

- Health checks across the consultation, payment, and notification services.
- Timer jobs, including the doctor-claim acceptance window and any escalation timers tied to mid-consultation tier changes.
- Baseline alerting so an operations team is notified of failed claims, stuck states, or payment reconciliation mismatches before a patient or doctor has to report them.

3.9 MVP Validation Checklist

The following end-to-end paths are implemented and should be treated as the current source of truth for what “live” means when this venture talks to investors, hospitals, or pilot users:

- Patients can create consultation requests for all three tiers.
- A doctor can atomically claim a request; a duplicate claim on the same request returns a conflict.
- A consultation can move through **REQUESTED → DOCTOR_ACCEPTED → IN_PROGRESS → COMPLETED**.
- A payment hold is created at request and captured at completion.
- Real-time and push pathways are triggered for both the request event and the doctor-acceptance event.

This checklist is a strong, concrete input for the “Now” / “Next 3 Months” milestones in the pitch deck's Traction & Milestones slide — the MVP claim/consultation/payment loop already works end-to-end ahead of the previously stated MVP-build timeline. Worth revisiting that slide alongside the financial plan.

3.10 Architecture Risk Notes

The capabilities above are real and demonstrable in the MVP, but a handful of implementation details determine whether they hold up past a small pilot. These were surfaced during technical review of the consultation console UI and are flagged here as engineering scoping input, not as defects in what has been built.

- **Atomic claim implementation.** The claim path should be a single conditional database update (e.g. UPDATE … WHERE status = 'REQUESTED') rather than a read-then-write sequence. A read-then-write leaves a race window where two doctors can both believe they've claimed the same request; a conditional update resolves the race at the database level with no separate lock required.
- **Durable claim-window timer.** The 2-minute acceptance window needs to survive a server restart or redeploy. An in-process timer does not; a delayed job queue or a polled expiry table does. Losing this silently means unclaimed requests never release their payment hold.
- **Side-effect orchestration for lifecycle transitions.** Each state change (claim, escalation, completion) triggers a payment operation, a notification, and potentially a biometric check. These should be coordinated through an outbox pattern or a workflow engine so a partial failure (e.g. payment captured but notification dropped) is caught and retried rather than silently invisible.
- **Independent payment ledger.** Nigerian payment gateways vary in whether they support a true authorization hold. An internal double-entry ledger (hold created, captured, released), reconciled against gateway webhooks, keeps the “not charged until care is delivered” claim auditable regardless of gateway behavior or downtime.
- **Communication provider abstraction.** Voice/video/chat vendor integration (Agora, Twilio, Vonage, or similar) should sit behind a thin interface so the consultation state machine never depends on vendor-specific session states. This keeps vendor switching or multi-vendor testing from touching core claim and payment logic.
- **Non-blocking biometric hooks.** Verification checks should run asynchronously and flag cases for review rather than gate the claim or session-start path — a slow or unavailable third-party verification vendor should never be able to delay a doctor from reaching an urgent patient.
- **NDPR-aligned data architecture.** Two decisions are cheaper to make now than retrofit later: where the primary database is hosted (data-residency expectations for health data), and

whether the audit trail is append-only by construction (an event log) rather than reconstructed from mutable status columns.

- **Instrumented SLA metrics from day one.** Claim-fill latency (REQUESTED to DOCTOR_ACCEPTED) and hold-release rate (requests that expire unclaimed) are the two numbers behind the speed-to-care claim in Section 6.1. Both should be emitted as first-class metrics from the first pilot session, not reconstructed from logs afterward.

These are risk flags for engineering scoping, not a complete technical specification. Each should be sized and prioritized against the pilot timeline before further platform build-out, and revisited once real usage data is available.

4. Hospital Partnership Strategy

On the provider side, the priority is converting hospital relationships into active use of the Mindset Healthcare platform and a reliable stream of patient referrals.

4.1 Objectives

- Get hospitals to adopt and actively use the platform.
- Get hospitals to send patient referrals and train their staff on the referral workflow.

4.2 Target List

Outreach will prioritize the top 50 hospitals in Nigeria, mapped against the procedures each performs, to ensure the provider network covers the specialties most in demand.

- Compile and prioritize the top 50 hospitals in Nigeria.
- Map each hospital's procedures and specialties to match against expected patient demand.

5. Diaspora Market Strategy

A parallel growth channel targets the Nigerian diaspora — encouraging Nigerians living abroad to use Nigerian hospitals for care, whether for themselves on visits home or for family members in-country.

5.1 Approach

- Identify and reach Nigerians in the diaspora through community and professional networks in each target market.
- Position the top 50 Nigerian hospitals and their procedures as the trusted destination network for diaspora patients.

5.2 Priority Diaspora Markets

Markets	Brazil, India, China, Philippines, Canada

These markets were flagged as priority diaspora targets in planning notes. Rationale (size of Nigerian community, remittance patterns, existing ties) should be added once available.

6. GTM Messaging Updates Driven by Platform Capabilities

The capabilities in Section 3 are not just backend milestones — they change what Portfolio A and Portfolio B campaigns (Section 2.4) can credibly claim. This section flags where channel messaging should be sharpened, without rewriting the full campaign tables in Section 2.4.

6.1 Patient-Facing Messaging (Portfolio A)

- Lead with tier choice, not just "book a doctor": patients can now see and choose Normal (₦1,000 voice-text), Priority (₦5,000 voice call), or Super Priority (₦10,000 video call) at the point of request.
- Speed-to-care becomes a provable claim rather than an aspiration, once pilot data confirms typical claim-fill time within the 2-minute doctor acceptance window — e.g. a CTA like "A doctor claims your request in minutes, not hours."
- Payment trust messaging: because payment is only captured on completion (Section 3.5), acquisition copy can honestly say patients are not charged unless care is delivered.

Any specific speed-to-care number (e.g. "average 90 seconds to claim") must come from real pilot data, not be invented for a campaign — treat this as an open item pending pilot metrics.

6.2 Clinician & Referrer-Facing Messaging (Portfolio B)

- Doctors can now be pitched on a concrete earnings mechanism, not just "join a network": claim consultation requests directly, at transparent per-tier rates (₦1,000 / ₦5,000 / ₦10,000), on a first-claim-first-served basis.
- The atomic claim system (Section 3.3) is itself a recruiting point for hospital administrators — it removes the double-booking and lost-referral friction that fragmented systems create today.
- Payment reliability messaging: doctors can be told the patient's payment is already held before they accept a case, reducing the classic no-show/non-payment worry that undermines trust in informal referral arrangements.
- Sample updated CTA for Facebook/LinkedIn Portfolio B: "Claim consultations in real time, get paid on completion — every time."

7. Open Items Before Finalization

- Confirm founder/team names and roles for the organizational chart (Section 1.3).
- Validate the campaign budget and CAC assumptions in Section 2.2 against real data.
- Finalize and attach the top 50 hospital list with procedures (Sections 4.2 and 5.2).
- Confirm the rationale and sizing for each priority diaspora market (Section 5.2).
- Validate the three tier price points (Section 3.2) against Nigerian willingness-to-pay data and reconcile with the financial plan.
- Decide and document the biometric verification rollout plan — which flows require it and whether it is mandatory or optional (Section 3.7).
- Capture real acceptance-window fill-rate and claim-time data from the pilot to support (or correct) the speed-to-care messaging in Section 6.1.
- Revisit the pitch deck's Traction & Milestones slide in light of the MVP Validation Checklist (Section 3.9) — the end-to-end claim/consultation/payment loop is further along than the current slide states.
- Scope engineering work against the architecture risk flags in Section 3.10 before further platform build-out.